

DEPRESSION AND/OR ANXIETY: ROUTINE CARE

Assess the patient with depression and/or generalised anxiety

Assess	When to assess	Note
Symptoms	Every visit	- Assess symptoms of depression and anxiety. If no better after 8 weeks of treatment or worse on treatment, discuss/refer. - Manage other symptoms as on symptom pages.
Self-harm	Every visit	Asking a patient about thoughts of self-harm/suicide does not increase the chance of this. If patient has suicidal thoughts or plans, assess and manage risk before continuing ↪ 83. Discuss with specialist before starting antidepressant.
Mania	Every visit	If abnormally happy, energetic, talkative, irritable or reckless, discuss/refer.
Anxiety	At diagnosis	- If excessive worry causes impaired function/distress for at least 6 months with ≥ 3 of: muscle tension, restlessness, irritability, difficulty sleeping, poor concentration, tiredness: generalised anxiety disorder likely. - If anxiety is induced by a particular situation/object, phobia likely, refer/discuss. - If repeated sudden fear with physical symptoms and no obvious cause, panic disorder likely, refer/discuss. - If previous bad experience causing nightmares, flashbacks, avoidance of people/situations, jumpiness or a feeling of detachment, post-traumatic stress disorder likely ↪ 88.
Dementia	At diagnosis	If for at least 6 months ≥ 1 of: memory problems, disorientation, language difficulty, less able to cope with daily activities and work/social function: consider dementia ↪ 148.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↪ 142.
Side effects	Every visit	Ask about side effects of antidepressant medication ↪ 145.
Stressors	Every visit	Help identify domestic, social and work factors contributing to depression or anxiety. If patient is being abused ↪ 88. If recently bereaved ↪ 86.
Family planning	Every visit	- Assess patient's contraceptive needs ↪ 154 - If patient pregnant or breastfeeding, doctor to discuss risks: the risk to baby from untreated depression may outweigh any risk from antidepressants. If possible, avoid antidepressants in first trimester of pregnancy. Ensure counselling/support and follow-up 2 weekly until stable. If possible, discuss with specialist.
Chronic conditions	Every visit	Ensure that other chronic conditions are adequately treated. If on oral steroids, efavirenz or atenolol, discuss with specialist.

Advise the patient with depression and/or generalised anxiety

- Explain that depression is a very common illness that can happen to anybody. It does not mean that a person is lazy or weak. A person with depression cannot control his/her symptoms.
- Explain that thoughts of self-harm and suicide are common. Advise patient that if s/he has these thoughts, s/he should not act, but tell a trusted person and return for help immediately.
- Educate the patient that anti-depressants can take 4-6 weeks to start working. Explain that there may be some side effects, but these usually resolve in the first few days.
- Emphasise importance of adherence even if feeling well. Advise patient that s/he will likely be on treatment for at least 9 months and it is not addictive. If difficulty with adherence, give support ↪ 173.
- Advise to avoid stopping treatment abruptly as patient may have withdrawal symptoms. If stopping, treatment needs to be tapered.
- Help the patient to choose strategies to get help and cope:

Get enough sleep

If difficulty sleeping ↪ 87.



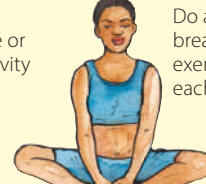
Encourage patient to take time to relax:



Spend time with supportive friends or family.



Find a creative or fun activity to do.



Do a relaxing breathing exercise each day.



Get active

Regular exercise might help.



Access support
Link patient with helpline or support group ↪ 178.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.

Health for All

Depression ↪ 96

Anxiety ↪ 100